
The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

A data-driven overview of global mental health conditions, their disease burden, and the persistent gap between need and treatment

SOURCE

Our World in Data / IHME Global Burden of Disease

DATA YEAR

2021

KEY INSIGHT

Mental health conditions affect hundreds of millions each year — depression alone will touch 1 in 3 women and 1 in 5 men over a lifetime.



WOMEN

1 in 3

will experience major depression
in their lifetime



MEN

1 in 5

will experience major depression
in their lifetime

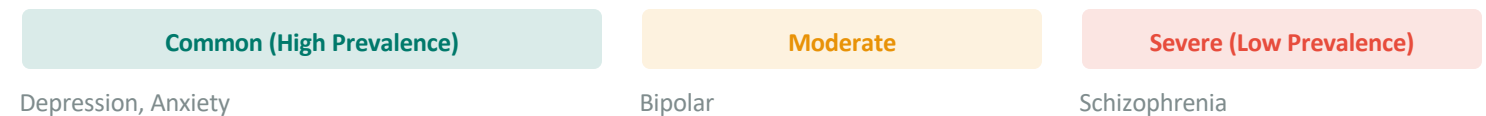
Why This Matters for Policy

- Mental health conditions span from **common** (depression, anxiety) to **less common but severe** (schizophrenia, bipolar).
- Hundreds of millions of people suffer from mental health conditions each year, and many more experience them over their lifetime.
- Poor mental health affects **well-being, ability to work, and relationships** with friends, family, and community.
- The scale of the problem demands a **systemic, policy-driven response** — not isolated interventions.

POLICY IMPLICATION

Lifetime prevalence this high means mental health must be integrated into universal health coverage, not treated as a specialty add-on.

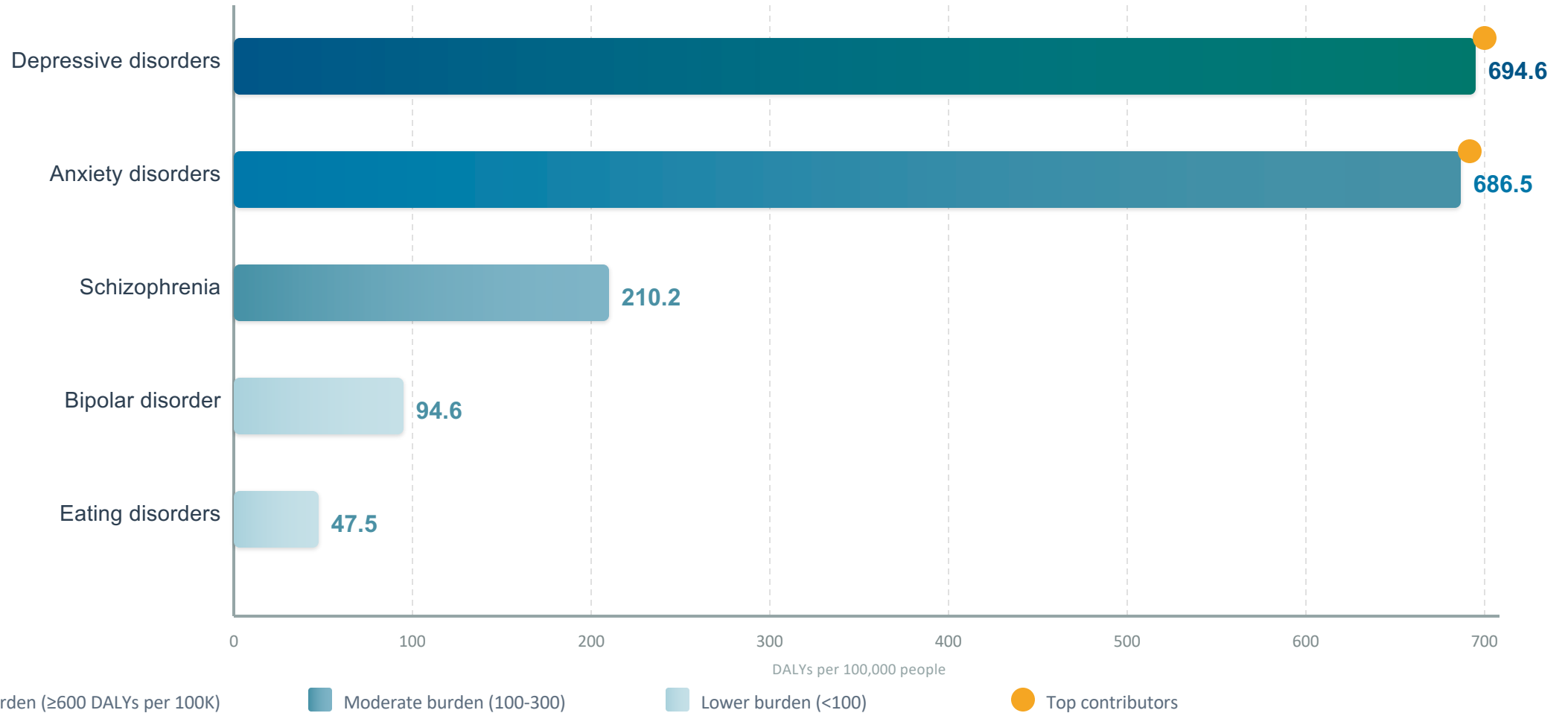
Spectrum of Mental Health Conditions:



Depression and Anxiety Dominate: ~695 and ~687 DALYs per 100K

Disease burden by category of mental illness, World, 2023 — DALYs per 100,000 people

DISEASE BURDEN



TAKEAWAY

Depression and anxiety together account for roughly **80%** of the total mental health disease burden across these five categories.

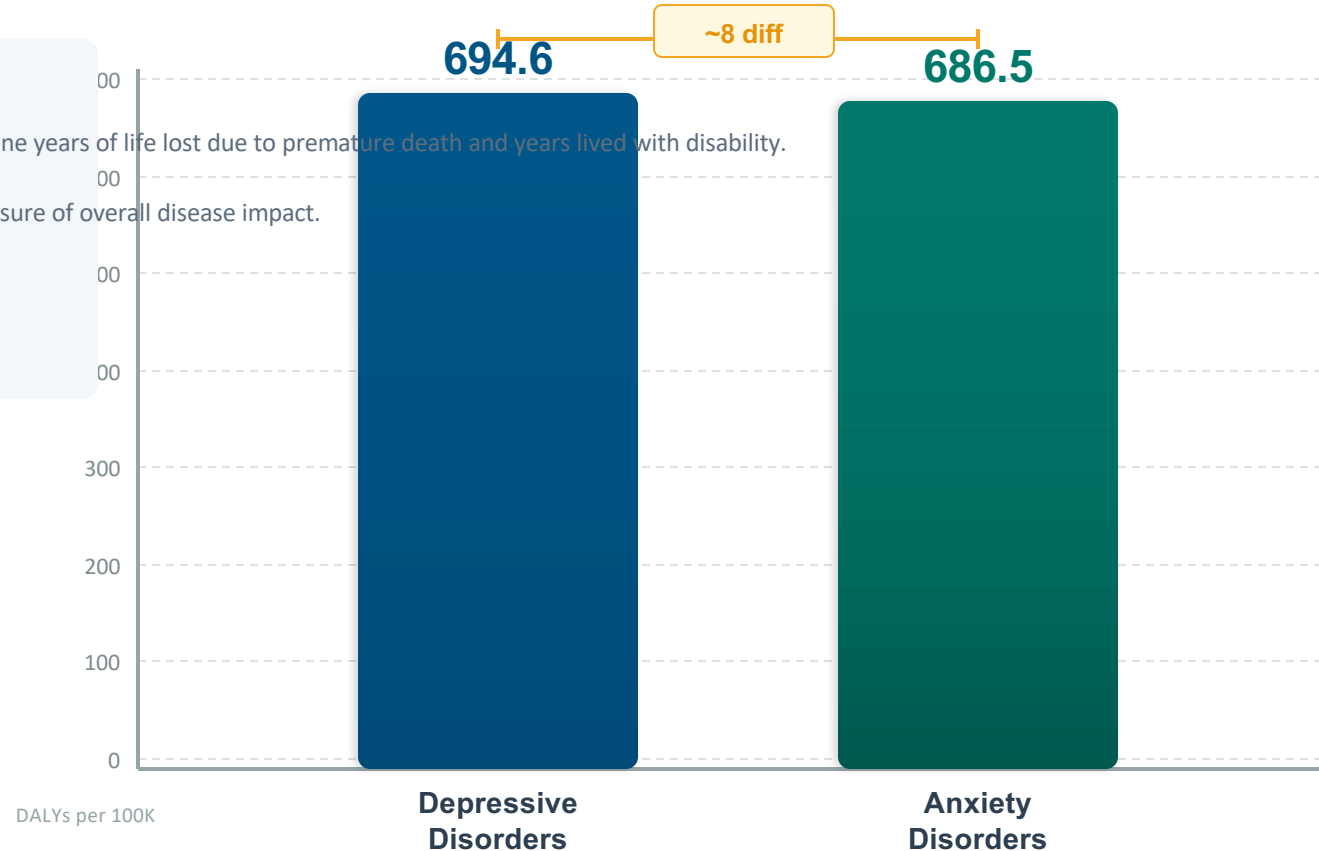
KEY INSIGHT A difference of only ~8 DALYs per 100K separates the two leading mental health conditions — they demand equal policy attention.

What are DALYs?

Disability-Adjusted Life Years combine years of life lost due to premature death and years lived with disability.

They provide a comprehensive measure of overall disease impact.

Higher DALYs = greater burden on population health.



Key Facts

- Difference of only **~8 DALYs per 100K** between the two conditions
- Together they account for **~80% of total** mental health DALYs
- Both are **common & treatable** yet severely under-addressed

POLICY NOTE

Parity in burden means policy responses must be **equally resourced.**

TAKEAWAY

Depression and anxiety together dwarf all other mental health categories in combined disease burden.

DALYs per 100,000 people — proportional comparison across all five categories, World, 2023

KEY INSIGHT

Population-level burden is lower, but individual severity and disability for schizophrenia and bipolar disorder are disproportionately high.

Depressive disorders

Anxiety disorders

Schizophrenia

210.2

Bipolar disorder

94.6

Eating disorders

47.5

Schizophrenia
210.2

DALYs per 100K

- Reflects severe disability associated with the condition
- Affects fewer people but with profound impact on daily functioning
- Requires long-term, specialized care and support systems

Bipolar Disorder
94.6

DALYs per 100K

- Meaningful but smaller share of population burden
- Episodic nature creates significant disability during episodes
- Effective treatment exists but requires consistent, long-term management

KEY INSIGHT

Mental illnesses are treatable and their impact can be reduced — yet significant barriers persist in access, quality, and willingness to seek care.



STIGMA

Social & Cultural Barrier

Many people are uncomfortable disclosing symptoms to healthcare professionals or even to people they know.

Result: Conditions go unreported and untreated



ACCESS

Infrastructure Barrier

Treatment is often lacking, especially in low- and middle-income countries where mental health infrastructure is minimal.

Result: Millions have no access to care



QUALITY

Standards Barrier

Even where treatment exists, it may be of poor quality — inadequate training, insufficient dosage, or premature discharge.

Result: Treatment received may not be effective



UNDERREPORTING

Data Barrier

Discomfort with disclosure makes it difficult to estimate true prevalence — the actual burden may be higher than reported figures suggest.

Result: True burden is likely underestimated

TAKEAWAY

Good data is essential to understand these conditions — how, when, and why they occur, and how they can be treated effectively and safely.

KEY INSIGHT

Coping strategies vary widely across countries — policy must account for cultural, economic, and healthcare system differences.

**Prescribed Medication****HIGHER-INCOME**

Common in higher-income settings where healthcare systems provide access to pharmacological treatment.

Evidence-Based

Most clinically validated approach to treating mental health conditions when properly prescribed and monitored.

Prevalence:

**High*****Religious / Spiritual****MANY REGIONS**

Prevalent in many regions worldwide — culturally embedded practices that provide meaning and community support.

Culturally Significant

Often the primary coping mechanism in societies where mental health care is less accessible.

Prevalence:

**High*****Talking to Friends/Family****WIDELY REPORTED**

A widely reported coping mechanism across cultures and income levels — social support as a first line of response.

Universally Accessible

Requires no formal system but effectiveness depends on social networks and stigma within community.

Prevalence:

**High***

*Relative prevalence based on global survey data — varies significantly by country and region. See figure_2.

**Depression and anxiety dominate the global mental health burden**

1

Together they represent the overwhelming majority of mental health DALYs — approximately 80% of the total across five major categories, each generating approximately 690 DALYs per 100,000 people.

**Severe conditions carry disproportionate individual burden**

2

Schizophrenia and bipolar disorder affect fewer people but create severe disability for individuals — schizophrenia contributes 210.2 DALYs per 100K, reflecting its profound impact on daily functioning and long-term outcomes.

**Treatment exists but access and quality remain inadequate**

3

A large treatment gap persists — mental illnesses are treatable, yet barriers in access (especially in low-income countries), quality of care, and willingness to seek help prevent effective intervention at scale.

**Stigma suppresses both help-seeking and accurate data**

4

Discomfort with disclosure means true prevalence is likely underestimated. Stigma operates as both a barrier to treatment and a distortion to the data that should guide policy decisions.

**Policy should prioritize scaling evidence-based interventions for common disorders**

5

while maintaining specialized care for severe conditions. The greatest population-level impact comes from scaling treatment for depression and anxiety — but severe conditions must not be neglected in resource allocation.